RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Usual adults dose: ***Analgesic or Antidysmenorrheal***-500 mg 3 times a day initially, followed by 250 mg every six hours as needed.

Note: It is recommended that mefenamic acid be used for no longer than 7 days at a time.

Usual pediatric dose: Children up to 14 years of age – Safety and efficacy have not been established.

Additional dosing information: It recommended that mefenamic acid therapy be discontinued promptly if diarrhea or a skin rash develops. Patients who develop diarrhea during mefenamic acid therapy are usually unable to tolerate the drug there after.

SYMPTOMS AND TREATMENT FOR OVERDOSE, AND ANTIDOTE(S):

Symptoms: Large doses produce excitement, incoordination , depression and convulsions in mice. Coma has also been reported.

Treatment: If ingestion is recent, forced emesis or gastric lavage should be performed and activated charcoal then given (0.5.g.kg), which may considerably reduce absorption. Careful observation during the first few hours is recommended, as convulsions are most likely to occur during this period. Very high serum levels may be associated with repeated convulsions which have been successfully suppressed with intravenous diazepam.

PACKING / PACK SIZES:

Blister packs of 10's, 2 x 10's/Box, 10 x 10's/Box and
100 x 10's/Box.

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS:

Store below 30°C in a tight container.

Auxiliary labelling

*Take with food. *Take with a full glass of water.
* May cause drowsiness. * Avoid alcoholic beverages.

SHELF LIFE: 3 years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER

SM PHARMACEUTICALS SDN BHD (218620-M)
Lot 88, Sungai Petani Industrial Estate
08000 Sungai Petani,
Kedah Darul Aman,
Malaysia.

22.04.2024

SM PHARMACEUTICALS SDN. BHD.

MEFCID (MEFENAMIC ACID) TABLET 500 MG

DESCRIPTION:

A yellow coloured, modified capsule shaped, film coated tablet with break-line on one side.

NAME(S) AND STRENGTH(S) OF ACTIVE INGREDIENT(S):

Each tablet contains: Mefenamic acid 500mg.

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Mefenamic acid is an analgesic with anti- inflammatory properties, and a demonstrable antipyretic effects.

Non-steroidal anti-inflammatory drug (NSAID) inhibit the activity of the enzyme cyclo-oxygenase, resulting in decreased formation of precursors of prostaglandins and thromboxanes from arachidonic acid. Although the resultant decrease in prostaglandin synthesis and activity in various tissues may be responsible for many of the therapeutic (and adverse) effects of NSAIDs, other actions may also contribute significantly to the therapeutic effects of these medications.

Analgesic -

May block pain impulse generation via a peripheral action that may involve reduction of the activity of prostaglandins, and possibly inhibition of the synthesis or actions of other substances that sensitise pain receptors to mechanical or chemical stimulation.

Antidysmenorrheal

By inhibiting the synthesis and activity of intrauterine prostagladins (which are thought to be responsible for the pain and other symptoms of primary dysmenorrhea), NSAIDs decrease uterine perfusion, and relieve ischemic as well as spasmodic pain. Also, NSAIDs may relieve certain to some extent. Extrauterine symptoms (such as headache, nausea,and vomiting) that they may be associated with excessive prostaglandin production.

Vascular headache prophylactic and suppressant

Analgesic actions may be involved in relief of headache. Also, by reducing prostaglandin activity, NSAIDs may directly prevent or relieve certain types of headache thought to be caused by prostaglandin-induced dilation or construction of cerebral blood vessels.

PM02092/05-24

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS):
Mefenamic acid is well absorbed after oral administration with peak plasma concentration occurring at 2 – 4 hours. The plasma half life is around 3 hours.

Mefenamic acid is extensively bound to plasma proteins (99%). Over 50% of a dose may be recovered in the urine, as unchanged drug or conjugated metabolites.

INDICATIONS:
Mild to moderate pain including muscular, traumatic and dental pain, headaches of most aetiology, post-operative and post-partum pain, pyrexia in children. Dysmenorrhea.

Mild to moderate pain in rheumatoid arthritis (including Still s Disease) and osteoarthritis.

Menorrheagia due to dysfunctional causes and presence of an IUD when other pelvic pathology has been ruled out.

CONTRAINDICATIONS:
1. Inflammatory bowel disease
2. Peptic and / or intestinal ulceration
3. Renal and hepatic impairment.
4. Asthma.

SIDE EFFECTS / ADVERSE REACTIONS:
The commonest adverse effects occurring with mefenamic acid are gastro-intestinal disturbances including diarrhoea. Peptic ulceration and gastro-intestinal bleeding have also been reported. Headache, drowsiness, dizziness and nervousness have been reported. There may be hypersensitivity reactions including skin rashes and urticaria, and occasionally allergic glmerulonephritis, asthma may be precipitated. Reported haematological effects haemolytic anaemia, agranulocytosis, pancytopenia and thrombocytopenia. Renal failure, glomerulonephritis and papillary necrosis have been reported.

Therapy should be discontinued if diarrhoea or skin rash occur.
Bronchospasm may be precipitated in patients suffering from, or with a previous history of, bronchial asthma or allergic disease.

Skin and subcutaneous tissue disorders:
Generalised bullous fixed drug eruption (GBFDE)

PRECAUTIONS / WARNINGS:

WARNINGS:

RISK OF GI ULCERATIONS, BLEEDING AND PERFORATION WITH NSAID:
Serious GI toxicity, such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g., dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.
Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g., alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less well than other individuals and account for most spontaneous reports for fatal GI events.

In patients suffering from dehydration and renal disease, particularly the elderly, concurrent therapy with other plasma protein binding drugs may necessitate a modification in dosage.

Pregnant women
The safety of mefenamic acid in pregnancy has not been established.

Breast milk
Very small amounts enter breast milk and may be transmitted to infants of nursing mothers.

The elderly
Both renal and haematological adverse effects appear to be more prevalent in the elderly. The minimum effective dose should be used and treatment stopped in the presence of diarrhoea or dehydration.

Skin Reactions
Serious skin reactions such as Generalised bullous fixed drug eruption (GBFDE) have been reported very rarely in association with the use of mefenamic acid. Mefenamic acid should be discontinued at the first appearance of the skin rash, mucosal lesions or any other sign of
hypersensitivity

DRUG INTERACTIONS:
Slight potentiation of oral anticoagulants is well documented, but this is rarely significant. When the drug is administered to patients receiving oral anticoagulant drugs, monitoring of prothrombin time as clinically indicated is necessary.